



Irritable bowel syndrome (IBS)

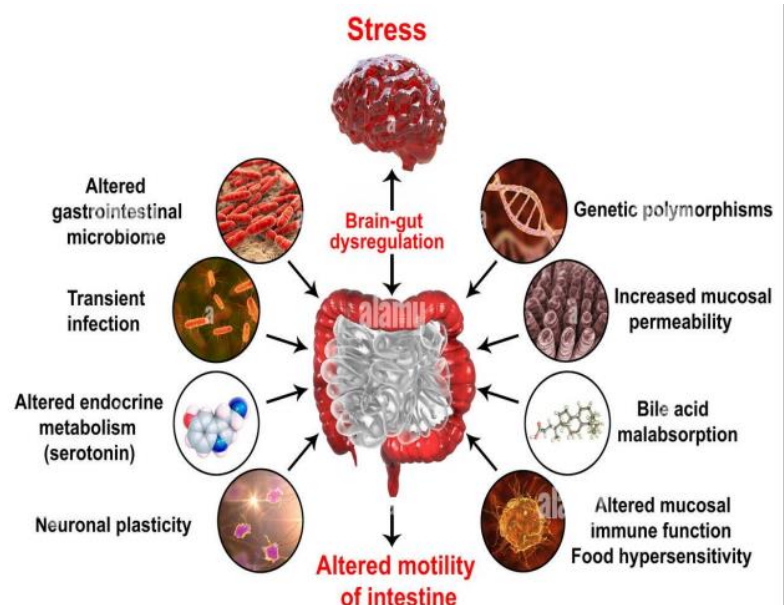
Is a common functional GIT disorder, characterised by recurrent abdominal pain in association with abnormal defecation in the absence of a structural abnormality of the gut. IBS accounts for frequent absenteeism from work and impaired quality of life. Young women are affected more often than men.



Pathophysiology

The cause of IBS is incompletely understood but biopsychosocial factors are thought to play an important role, along with luminal factors, such as diet and the gut microbiota.

- Behavioural and psychosocial factors
- Physiological factors
- Luminal factors





1. Behavioural and psychosocial factors

Early learning difficulties or emotionally challenging* .interactions during childhood may contribute to IBS in later life about 50% of patients referred to hospital have a psychiatric* .illness, such as anxiety, depression, somatisation and neurosis Acute psychological stress and overt psychiatric disease are* .known to alter visceral perception and gastrointestinal motility .**These factors contribute to but do not cause IBS**

2. Physiological factors

IBS is thought to be a disorder of the brain–gut axis, with alterations .in visceral hypersensitivity There is some evidence that IBS may be a serotonergic (5-HT)* disorder, as evidenced by relatively excessive release of 5-HT in diarrhoea-predominant IBS (IBS-D) and relative deficiency with . constipation-predominant IBS (IBS-C). (treatment ?!) There is some evidence that IBS may represent a state of low- * .grade gut inflammation or immune activation There may be raised numbers of mucosal mast cells that sensitise * .enteric neurons by releasing histamine and tryptase

3. Luminal factors

Both quantitative and qualitative alterations in* intestinal bacterial microbiota have been reported. Small intestinal bacterial overgrowth (SIBO) may be present in some patients and lead to symptoms. This ‘gut dysbiosis’ may explain the response to probiotics .or the non-absorbable antibiotic rifaximin Dietary factors are also important. Some patients* .have chemical food intolerances (not allergy)



Clinical features

The key symptoms of IBS include recurrent* .abdominal pain and altered bowel habit dx. Depends on Rome IV criteria *

i 23.74 Rome IV criteria for diagnosis of irritable bowel syndrome

Recurrent abdominal pain at least 1 day per week on average in the last 3 months (onset at least 6 months before diagnosis), associated with *two or more* of the following:

- Related to defecation
- Onset associated with a change in frequency of stool
- Onset associated with a change in form (appearance) of stool

Clinical features –cont

Abdominal pain is usually colicky or cramping in nature,* .felt in the lower abdomen and related to defecation The bowel habit is variable, with IBS stratified by* predominant bowel habit; those with mainly constipation (IBS-C), mainly diarrhoea (IBS-D), mixed bowel habit (IBS-M) or unsubtyped (IBS-U) Those with constipation tend to pass infrequent pellety* stools, usually in association with abdominal pain or .proctalgia

Clinical features-cont

Those with diarrhoea have frequent defecation but produce low-volume stools and rarely have nocturnal symptoms. Passage .of mucus is common but **rectal bleeding does not occur** Patients **do not lose weight** and are **constitutionally well. (the dx of IBS is unlikely if alarming features are present)** other non GIT symp. May be present like (migraine , dyspareunia, * .cystitis & psychological illnesses) Onset after gastroenteritis may point towards a diagnosis of post- * .infectious IBS Physical examination is generally unremarkable, with the exception of * .variable tenderness to palpation

i 23.75 Alarm features in suspected irritable bowel syndrome

• Age >50 years	• Family history of colon cancer or inflammatory bowel disease
• Unintentional weight loss	• Anaemia
• Nocturnal symptoms	• Evidence of overt gastrointestinal bleeding (i.e. melaena or fresh blood in stools (haematochezia))
• Recent change in bowel habit	
• Palpable abdominal mass or lymphadenopathy	





Diagnosis

Diagnosis is made through a careful history,* exploring diet, medical, surgical and psychological .history The diagnosis is clinical in nature and can be made* confidently in most patients using the Rome IV criteria, combined with the absence of alarm symptoms and without resorting to complicated .tests

.dx.- cont

Few recommended test can be performed to exclude other * :conditions

.1 .Cbp , ferritin

.2 . TSH (IBS -D)

.3 .CELIAC screen.

All should be normal in IBS .3

.4 .Fecal calprotectin

.5 . GSE (IBS-D)

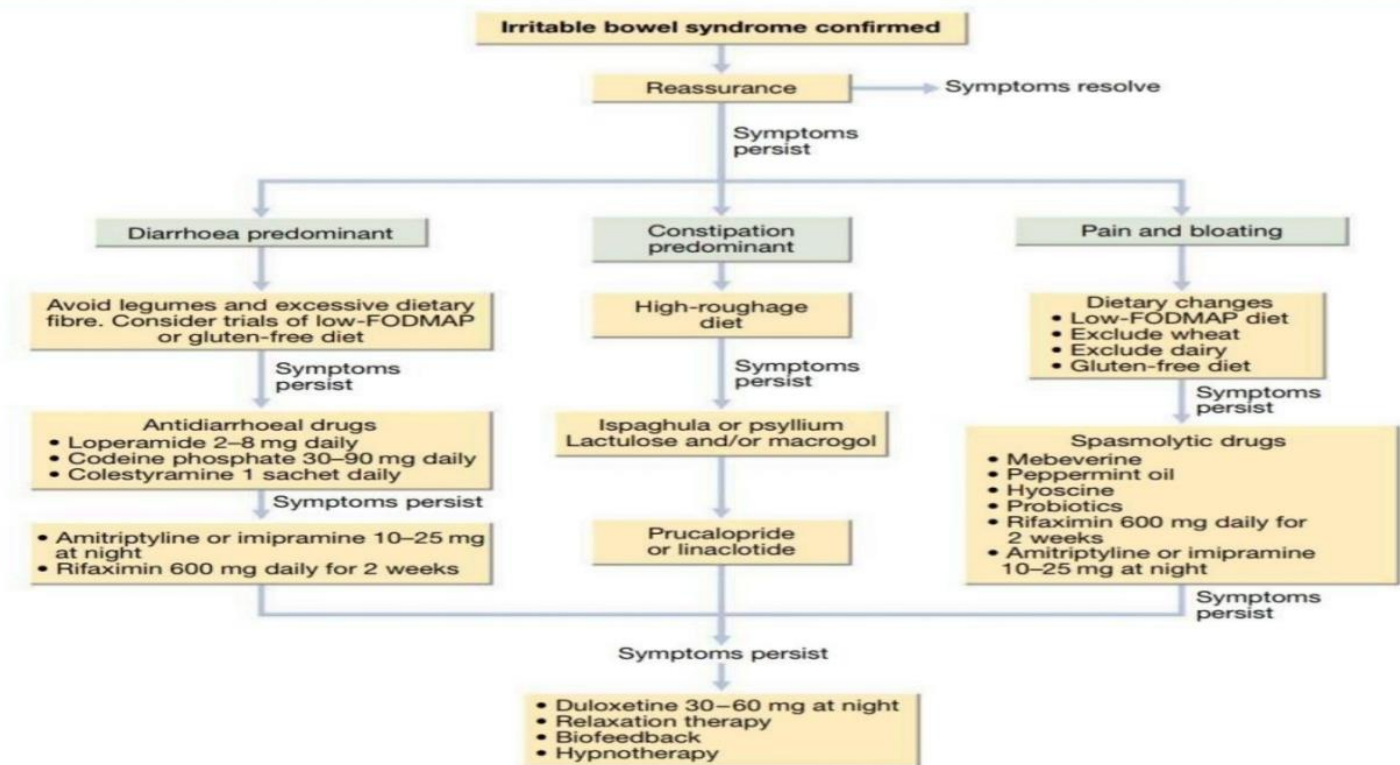
.6 If there are alarming features : **colonoscopy** should be done (IBD, CA colon)

Management

The most important steps are to make a positive* .diagnosis, as well as educating and reassuring the patient Treatment is traditionally tailored to the predominant* .symptoms Dietary modification is very important (See the table in* .the next slide) Probiotics, in capsule form, may be effective if taken for* several months, although the optimum combination of .bacterial strains and dose have yet to be clarified Patients with intractable symptoms sometimes* bene t from several months of therapy with a □ tricyclic antidepressant, such as **amitriptyline** or **imipramine** (It may act by reducing visceral).sensation and by altering gastrointestinal motilit IBS- C may respond to **prucalopride**, **linaclotide** &* .**lubiprostone** . IBS –



D may respond to loperamide & Rifaximin * Abdominal colic can be treated with mebeverine or* .Hyoscine



23.67 Management of irritable bowel syndrome. (FODMAP = fermentable oligo-, di- and monosaccharides, and polyols)

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23.76 Dietary management of irritable bowel syndrome

- Eat regularly and avoid missing meals
- Take time to eat
- Ensure adequate hydration and avoid carbonated and caffeinated drinks
- Reduce alcohol intake
- Reduce intake of 'resistant' starch and insoluble fibre
- Avoid foods with artificial sweeteners
- Consider a low FODMAP diet
- Consider a gluten-free diet
- Consider a lactose exclusion diet

(FODMAPs = fermentable oligo-, di- and monosaccharides, and polyols)

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23.77 Complementary and alternative therapies for irritable bowel syndrome

Manipulative and body-based

- Massage, chiropractic, yoga*

Mind–body interventions

- Meditation, hypnosis*, cognitive therapy

Biologically based

- Herbal products*, dietary additives, probiotics*

Alternative medical systems

- Ayurveda, homeopathy, traditional Chinese medicine

*Some evidence for benefit exists.

**To Summarize**

IBS is a common functional GI disorder *.Dx can be made clinically by Rome IV criteria * There should be NO alarming features in patients*with IBS .Treatment should be individualized *